

Communication and Substance Use

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September 25, 2025



Objectives

- Identify screening tools used to screen for substance use disorder
- Analyze a CURES report
- Produce a Controlled Substance Agreement
- Practice a brief intervention based on the SBIRT model

Pain: The "Fifth Vital Sign"

Pain Standards of the Joint Commission on Accreditation of Healthcare Organizations

- Recommendation: make pain assessment/management a priority in daily practice
- Consider pain intensity the fifth vital sign: measure along with temperature, pulse, respiration, and BP
- Patients' rights: full pain work-up when pain is not easily characterized or treated

JCAHO, 1999-2000.

4

FREEDOM FROM PAIN!

Extra strength pain relief free of extra prescribing restrictions.

- Telephone prescribing in most states
- Up to five refills in 6 months
- No 30-minute Rx required

Excellent patient acceptance.

The benefits of VICODIN^{ES} over a full-strength opioid.

vicodin^{ES}

Tablet for tablet, the most potent analgesic you can phone in.

Maintain control of your patient's therapy.

Rx Specify *Do not substitute*

vicodin^{ES}

It's your prescription—not a suggestion.

A little history on how we got here...

FOR MODERATE TO SEVERE MALIGNANT OR POST-OPERATIVE PAIN

going on holiday
the family
doing the garden
walking the dog
sundays
fish and chips

Let them focus on the things that matter

OxyContin[®]

Prescription Oxycodone Extended-Release Tablets

When you know NSAIDs or acetaminophen will not be enough

OxyContin[®] q12h

• Rapid onset of analgesia within 46 minutes^{1,2}

• Full 12 hours of pain relief^{1,2}

• No risk of acetaminophen or ASA toxicity^{1,2}

Take the next step in pain relief

OxyContin[®] q12h 15 mg, 20 mg, 40 mg, 80 mg

One to Start and Stay With... Easy to Dose, Easy to Titrate

Opioid Prescribing Rates

Figure 1: Annual Opioid Analgesic Prescriptions by Drug Substance, 1992-2017

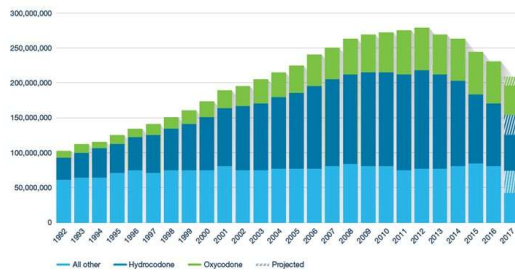
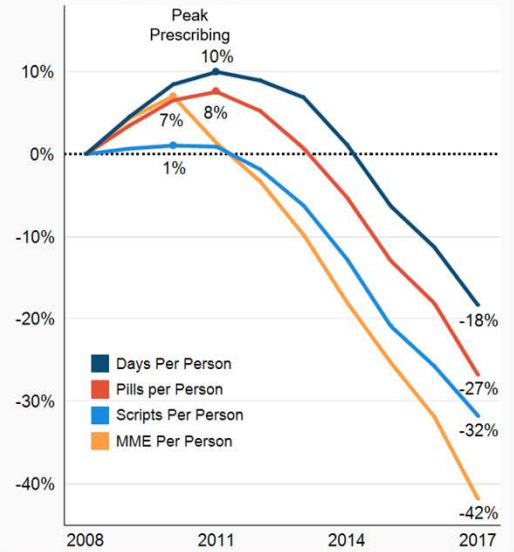


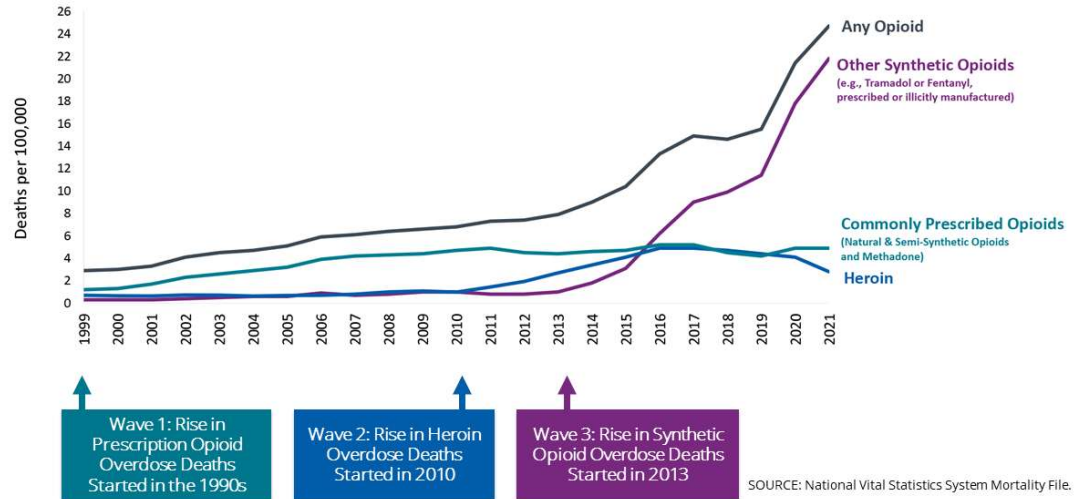
Figure 1: National Percent Change in Prescription Opioid Use since 2008



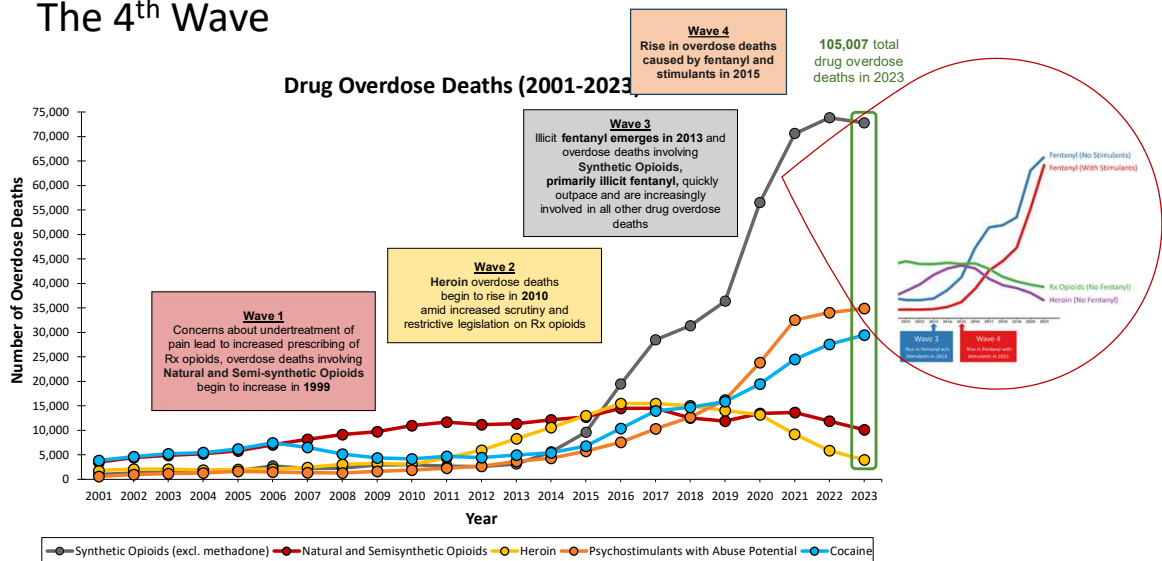
1996 – oxycotin approved

The Third Wave

Three Waves of Opioid Overdose Deaths



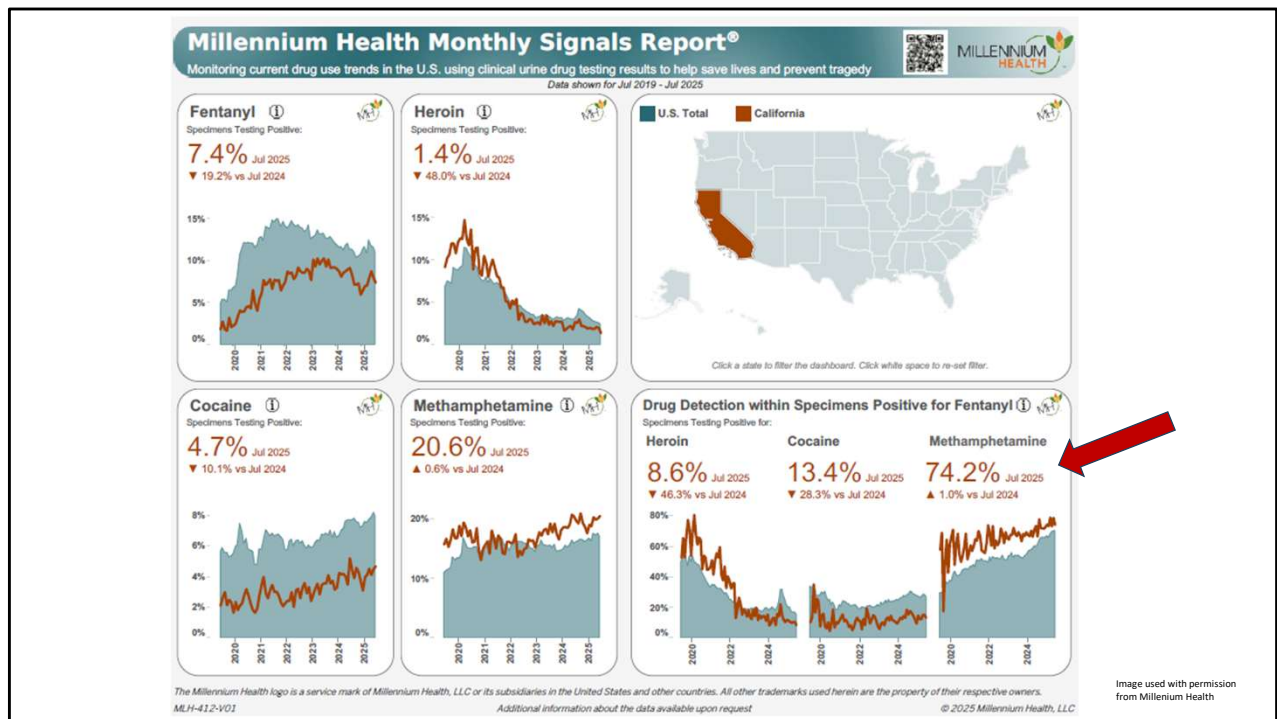
The 4th Wave



1. Spencer MR, Miniño AM, Warner M. Drug overdose deaths in the United States, 2001–2021. NCHS Data Brief, no 457. Hyattsville, MD: National Center for Health Statistics. 2022. DOI: <https://dx.doi.org/10.15620/cdc:122556>.
2. Spencer MR, Garnett MF, Miniño AM. Drug overdose deaths in the United States, 2002–2022. NCHS Data Brief, no 491. Hyattsville, MD: National Center for Health Statistics. 2024. DOI: <https://dx.doi.org/10.15620/cdc:135849>.
3. Garnett MF, Miniño AM. Drug overdose deaths in the United States, 2003–2023. NCHS Data Brief, no 522. Hyattsville, MD: National Center for Health Statistics. 2024. DOI: <https://dx.doi.org/10.15620/cdc:170565>

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Most are probably familiar with the 3 waves of the opioid epidemic. First there was concern about undertreatment of pain that led to overprescribing of opioids. Then we saw the rise in heroin and eventually synthetic opioids, primarily fentanyl, as it was cheaper to obtain and more potent. Now the 4th wave is the trend observed in the last decade with the rise in concurrent stimulant use with fentanyl. The gray line represents OD deaths that involved fentanyl. The orange line represents OD deaths involving stimulants. But the reality is there is a lot of overlap between those two lines. We are increasingly seeing many OD deaths that involved both substances together represented by the red line in this breakout graph. Stimulants are being used to counter the sedating effects of the fentanyl. And fentanyl is being added to stimulants to create added euphoria and opioid dependency that help retain customers for drug dealers.



These graphs show some of the trends in CA (orange line) and nation wide (blue) among urine drug specimens collected by Millennium Health. Specifically pay attention to the bottom right corner. Among all the specimens testing positive for fentanyl, there has been an alarming increase in methamphetamine positivity.

CDC's Clinical Practice Guideline for Prescribing Opioids for Pain

[Print](#)

2022

CDC Clinical Practice Guideline for Prescribing Opioids for Pain



CLINICAL PRACTICE GUIDELINE AT A GLANCE



GUIDELINE RECOMMENDATIONS AND GUIDING PRINCIPLES



WHAT'S DIFFERENT

The 12 recommendations are grouped into four areas of consideration.



[Determining whether or not to initiate opioids for pain](#)

(Recommendations 1, 2)



[Deciding duration of initial opioid prescription and conducting follow-up](#)

(Recommendations 6, 7)



[Selecting opioids and determining opioid dosages](#)

(Recommendations 3, 4, 5)



[Assessing risk and addressing potential harms of opioid use](#)

(Recommendations 8, 9, 10, 11, 12)

<https://www.cdc.gov/overdose-prevention/hcp/clinical-guidance/index.html>

<https://www.cdc.gov/mmwr/volumes/71/rr/rr7103a1.htm>

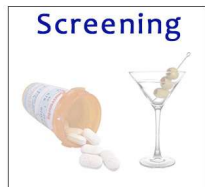
<https://www.cdc.gov/overdose-prevention/hcp/clinical-guidance/recommendations-and-principles.html>

Steps to Initiate when Starting and Continuing a Controlled Substance

1. Consider if there are safer alternative treatment options?
2. Screen for patient's current level of substance use risk (SBIRT) and inquire about prior history of substance use disorder
3. Utilize Prescription Drug Monitoring Program (CURES in California)
4. Run point of care urine drug screen
5. Complete a Controlled Substance Agreement with the patient

SBIRT

Screening, Brief Intervention
and Referral to Treatment



High Risk Use

Severe Use

Brief
Intervention

Referral to
Treatment

Annual questionnaire

Once a year, all our patients are asked to complete this form because drug and alcohol use can affect your health as well as medications you may take. Please help us provide you with the best medical care by answering the questions below.

Patient name: _____
Date of birth: _____

Are you currently in recovery for alcohol or substance use? ☐ Yes ☐ No

Alcohol: One drink =  12 oz. beer  5 oz. wine  1.5 oz. liquor (one shot)

	None	1 or more
MEN: How many times in the past year have you had 5 or more drinks in a day?	☐	☐
WOMEN: How many times in the past year have you had 4 or more drinks in a day?	☐	☐

Drugs: Recreational drugs include methamphetamines (speed, crystal), cannabis (marijuana, pot), inhalants (paint thinner, aerosol, glue), tranquilizers (Valium), barbiturates, cocaine, ecstasy, hallucinogens (LSD, mushrooms), or narcotics (heroin).

	None	1 or more
How many times in the past year have you used a recreational drug or used a prescription medication for nonmedical reasons?	☐	☐

<https://www.sbirt.care/tools.aspx>

- Two key reasons that screening should be universal are that **alcohol and drug use are common** and that **substance use often goes undetected**.

Substance Use Disorder Assessment Tools

Tool	Substance type		Patient age		How tool is administered	
	Alcohol	Drugs	Adults	Adolescents	Self-administered	Clinician-administered
Tobacco, Alcohol, Prescription medication, and other Substance use (TAPS)	X	X	X		X	X
CRAFT 🔗	X	X		X	X	X
Drug Abuse Screen Test (DAST-10)* <i>For use of this tool - please contact Dr. Harvey Skinner 📧</i>		X	X		X	X
Drug Abuse Screen Test (DAST-20: Adolescent version)* <i>For use of this tool - please contact Dr. Harvey Skinner 📧</i>		X		X	X	X
NIDA Drug Use Screening Tool (NMASSIST) <i>(discontinued in favor of TAPS screening above)</i>	X	X	X			X
Alcohol Screening and Brief Intervention for Youth: A Practitioner's Guide (NIAAA)	X			X		X

<https://nida.nih.gov/nidamed-medical-health-professionals/screening-tools-resources/chart-screening-tools>

Prescription Drug Monitoring

CURES (Controlled Substance Utilization Review and Evaluation System) is a database of Schedule II, Schedule III, Schedule IV and Schedule V controlled substance prescriptions dispensed in California serving the public health, regulatory oversight agencies, and law enforcement.

Which State PDMP(s) Do You Want to Search?

Patient records may be searched from the Prescription Drug Monitoring Programs (PDMPs) of the states listed below.

- ☐ Select All Available States
- ☐ **Bordering States**
- ☐ Arizona (AZ) coming soon
 - ☐ Nevada (NV) coming soon
 - ☐ Oregon (OR) coming soon

The screenshot shows the CURES (Controlled Substance Utilization Review and Evaluation System) interface. At the top, there is a navigation bar with links: HOME, PATIENT ACTIVITY, RX FORM THEFT/LOSS, and MY PROFILE. Below the navigation bar, the text 'Welcome to CURES' is displayed. The main section is titled 'Patient Activity Search'. It includes a 'Your Saved Searches' dropdown menu with 'Select one' as the current selection. Below this, there is a search criteria section. It starts with '(or)' and a note: 'You must enter either a partial Last Name or First Name, and the exact Date of Birth in order to search patient records.' There are three input fields: 'Last Name', 'First Name', and 'Date of Birth'. The 'Date of Birth' field has a placeholder 'MM/DD/YYYY' and a note '(and) Must be exact:'. Below the input fields, there is a checkbox for 'Select All Available States' and a section for 'Bordering States' with three options: 'Arizona (AZ) coming soon', 'Nevada (NV) coming soon', and 'Oregon (OR) coming soon'. At the bottom of the search section, there is a note: 'By executing this search, I understand the Terms and Conditions governing CURES access and use.' Below this note are two buttons: 'Search' and 'Clear'.

Note – methadone dispensed at an opioid treatment program will NOT appear on a CURES report

The California Department of Justice (DOJ) has entered into an interstate data sharing agreement with the state of Oregon as of May 8, 2023.

Effective October 2, 2018, with specified exceptions, health care practitioners authorized to prescribe, order, administer, or furnish a controlled substance shall consult the CURES database to review a patient's controlled substance history no earlier than 24 hours, or the previous business day, before prescribing a Schedule II, Schedule III, or Schedule IV controlled substance to the patient for the first time and at least once every 6 months thereafter if the substance remains part of the treatment of the patient. (Health and Safety Code section 11165.4(a)(1)(A)(i))

Patient Activity Report

Prescribers Associated with Patient Identities | Peer-to-Peer Messaging



Patient Identities Selected: 1



PRESCRIPTION RECORDS FOUND: 13

Search Results



Prescription Information						Prescription Numbers		
Patient Details ▾	Dates and Payment ▾ ⓘ	Drug Name ▾ Refills ▾	Strength ▾ Form ▾	Days Supply ▾ Quantity ▾	Daily MMEs ▾ Total MMEs ▾	Prescriber Details ▾	Pharmacy Details ▾	Serialized Rx ▾ Pharmacy Rx ▾
	Filled: 08/26/2023 Sold: 08/26/2023	HYDROCODONE BITARTRATE, ACETAMINOPHEN Refill 0 of 0	10-325 MG TAB	Days: 30 Qty: 90	Daily: 30.0 Total: 900.0	KINNEVEY, CHRISTINA, E. (MD) DEA: FK5572703	WALGREENS #02575 Pharmacy #: PHY52797	Serialized Rx: NOT REPORTED Pharmacy Rx: 1919075
	Filled: 07/26/2023 Sold: 07/28/2023	HYDROCODONE BITARTRATE, ACETAMINOPHEN Refill 0 of 0	10-325 MG TAB	Days: 30 Qty: 90	Daily: 30.0 Total: 900.0	KINNEVEY, CHRISTINA, E. (MD) DEA: FK5572703	WALGREENS #02575 Pharmacy #: PHY52797	Serialized Rx: NOT REPORTED Pharmacy Rx: 1912030

Expand All
≡ ≡ ≡



Point of Care Urine Drug Screen

- HOW LONG - DO DRUGS STAY IN THE BODY?					
FACTORS THAT INFLUENCE TIME IN BODY					
Body Mass	Type of Drug/ Amount used	Physical Activity	Water in the body	Frequency of use	Tolerance
DRUG NAME	SALIVA	BLOOD	URINE	HAIR	
Heroin	1 DAY	5 - 24 HOURS	1 - 2 DAYS	3 MONTHS	
Marijuana	3 DAYS	1-2 DAYS	3 DAYS	3 MONTHS	
Cocaine	2 DAYS	2 DAYS	3 - 14 DAYS	3 MONTHS	
Meth	2 DAYS	25 HOURS	1 - 5 DAYS	3 MONTHS	
Adderall	20 - 50 HOURS	46 HOURS	2 - 3 DAYS	3 MONTHS	
Fentanyl	N/A	5 - 48 HOURS	1 - 3 DAYS	3 MONTHS	
Hydrocodone	12 - 36 HOURS	1 DAY	4 DAYS	3 MONTHS	
Oxycodone	1 - 4 HOURS	1 DAY	3 - 4 DAYS	3 MONTHS	
LSD	12 - 24 HOURS	6 - 12 HOURS	8 HOURS	6 MONTHS	
MDMA	1 - 2 DAYS	1 - 2 DAYS	1 - 3 DAYS	3 MONTHS	

Urine Drug Screens

- Often need to ignore the quantified level of substance reported on test results (does not correlate with amount used). Level of drug impacted by time of last use and individual metabolism
- When in doubt, call the lab to ask questions!

Agent	Summary of Agents Potentially Contributing to False Positives ³⁻⁶			
Marijuana	• dronabinol	• NSAIDs*	• hemp foods: tea, oil ⁷	
Metabolites	• efavirenz	• proton pump inhibitors		
Cocaine Metabolites	• coca leaf teas	• topical anesthetics containing cocaine		
Opioid Metabolites	• dextromethorphan	• levofloxacin	• poppy seeds	• rifampin
	• fluoroquinolones	• ofloxacin	• poppy oil	• quinine
Amphetamines/ Methamphetamine (High Rate of False Positives)	• amantadine	• dextroamphetamine	• labetalol	• ranitidine
	• benzphetamine	• doxepin	• l-methamphetamine	• selegiline
	• brompheniramine	• ephedrine	• (OTC nasal inhaler)	• phenyl-propanolamine
	• bupropion	• fluoxetine	• methylphenidate	• promethazine
	• chlorpromazine	• isometheptene	• MDMA	• pseudoephedrine
	• desipramine	• isosuprine	• phentermine	• trimethoprim
Benzodiazepines	• oxaprozin	• sertraline		
Barbiturates	• ibuprofen	• naproxen		
Methadone	• chlorpromazine	• doxylamine	• thioridazine	
	• clomipramine	• ibuprofen	• verapamil	
	• diphenhydramine	• quetiapine		
Alcohol	• mouthwash	• short-chain alcohols	• OTC cough products (isopropyl alcohol)	

* NSAIDs resulting in false-positive for marijuana mainly consist of ibuprofen and naproxen and modern tests do not result in false positives; ⁷ THC concentrations in hemp products are low enough to prevent positive immunoassay results.

<https://freebythesea.com/addiction-resources/how-long-drugs-in-system/>

Substance Dependence vs. Substance Use Disorder (SUD)

- **Language matters! Dependence $\neq$ SUD!**
- **Physical dependence**
 - Refers to the physiological changes that occur with taking a substance (i.e tolerance), and the withdrawal symptoms that occur with cessation



"Words are important. If you want to care for something, you call it a 'flower'; if you want to kill something, you call it a 'weed'."
- Don Coyhis

The term addiction is no longer favored to describe substance use disorders due to the negative associations, stigma and lack of precision

Controlled Substance Agreements



- Formerly called Pain or Narcotic contracts
- Outline “rules” regarding prescribing controlled substances (opiates, benzos, ADHD meds, etc)
- Agreement versus legal contract

Case Study – 25 minutes

- Work with a student partner to review 1 of the 3 cases posted on Canvas. One student partner chooses to be the doctor and one the patient (case numbers will be assigned in class)
- Discuss the two discussion questions with your partner
- Role play and fill out the Controlled Substance Agreement. Each student should upload a copy of the same agreement to the assignment page

Large group discussion on each case

- 15 minutes

1 month later...

You obtain an office urine drug screen (UDS) on each of these patients.

Henry Rafter's UDS shows hydrocodone as well as diazepam

Lisa Freidman's UDS shows amphetamines as well as methamphetamines.

Luis Vargus' UDS is negative for all substances including oxycodone.

Discuss with your student partner what additional questions you would ask your patient and how you would proceed based on these results (5 minutes followed by large group discussion for 10 minutes)



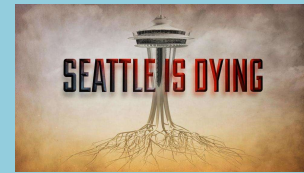
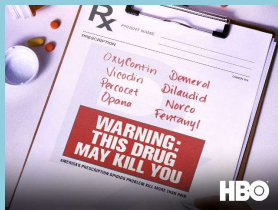
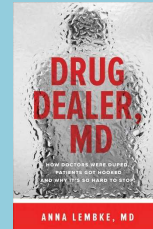
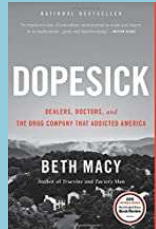
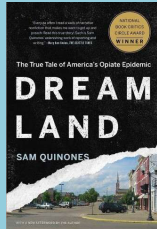
- Solano County's Opioid Safety Coalition
- Team of community members working to improve opioid safety in the community
 - Doctors, counselors, pharmacists, nurses, educators, public health, law enforcement, nonprofit organizations, concerned residents
- Started in 2018
- Meets monthly
- <http://www.drugsafesolano.org/>



<https://www.facebook.com/SupervisorErinHannigan/photos/a.428808347250938/1682273441904416/?type=3&theater>

DSS is a multidisciplinary coalition working under the California Department of Public Health. It was founded in late 2018. It was made as a response to the opioid epidemic in Solano County. We aim to increase awareness about opioids and promote safe use.

Reading & Documentary Recommendations



Beautiful Boy
Ben is Back